

Swiss Health Insurance Analysis

PP221: Fundamentals of Public Policy Assignment

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Pick a policy that you think has worked well.

We are going to analyse the Bundesgesetz über die Krankenversicherung (KVG)[1], or more simply the Swiss Federal Health Insurance Law.

KVG's primary goal was to ensure universal affordable healthcare access. Its secondary goal was to promote solidarity[2]. The ranked characterisation is mine, the law does not explicitly state these.

- KVG mandates all Swiss residents to take up a medical insurance (Art. 3).
- Everyone is charged an equal premium (Art. 61). This is an oversimplified sentence. You can differentiate based on regions, but the differences are regulated. You cannot differentiate on age among adults.
- KVG explicitly states the freedom and mechanics of changing insurers (Art. 7).

...identify 3 things that you think explain why it worked well.

1. If all adults pay the same premium, insurers face more cost if they have a higher risk base. You will have an incentive to prioritise healthy young people. But you cannot do that either (Art. 3). How does an insurer survive then? There is a risk equalisation framework in place (Art. 16-18). If you are an insurer with a lower cost than average, you pay into a pool. If you are an insurer with a higher cost than average, you get paid out by the pool. This explicitly dampens the incentive for insurers to either charge differentiated premium or refuse higher risk customers.

The risk equalisation existed for a few years as an emergency federal decree[3] already to serve this purpose and KVG formalised it as law. This policy works well because it comprehensively addresses the incentives for the market participants. It proactively alleviates the pressure that would arise due to flat pricing alone.

The policy is most stringent on basic insurance. Insurers are allowed to promote and price supplementary products. I like this detail, as it leaves insurers a residual profit incentive to operate at all.

2. Switzerland has the state capacity to enforce this law. This policy is laden with things that the State must do. You need to operate the joint institution for risk equalisation, Gemeinsame Einrichtung KVG (GE KVG). The policy also requires the State to decide the schedule of costs for basic healthcare (KLV, Art. 25-26). You need to enforce universal enrollment. The State also needs to track insurer solvency, not only out of goodness of its heart, but because the joint institution has counterparty risk from each insurer. All of these functions require intensive data collection to be effective.
3. The policy promotes solidarity. This is important to the Swiss people, and if health is a domain where the State should intervene, then equalising premiums across the population is a direct move to support that. The policy works well because it aligns well with politics. Based on the post-referendum survey [2], solidarity was the top reason to support this policy among Yes voters.

...identify at least 1 thing that could be done differently to make it work better.

1. Healthcare is considered a necessity. The cost of healthcare is a matter of public welfare. Thus, an equal premium mandate can be difficult for people with fewer means. KVG has a provision for cantons to subsidise premium payments (Art. 65). This comes with a subsidy floor for some sections of society such as children and students, and a federal contribution to the cantons. The canton of Vaud[4], for example, imposes a cap of 10% of income on your premium payments.

My proposal is to simplify the federal policy to an income cap. Cantons deciding their own subsidy frameworks creates political friction. Alas, in 2024[5], this was put to referendum and failed.

2. A natural tension for any healthcare insurance policy is the demographics of the society. The working age population can pay more and needs less healthcare as compared to the elderly. People will have a natural tendency to feel they are overpaying for others.

I propose that a small portion of my premium payment can go towards a pool of money held by the joint institution. When I am older, I should be able to withdraw money from this pool. This is exactly how the joint institution works for insurers at fixed points of time averaged over people. But it can also work for fixed people averaged over time. Germany has a form of this, but their reserves are held with insurers with a cost imposed on switching[6]. Switzerland's joint institution is better suited to handle this directly.

Identify at least one possible unintended consequence of this policy. Any thoughts on how to avoid or mitigate this? Would the things done to avoid or mitigate it have unintended consequences of their own?

The profit incentive for running an insurance company is difficult to gauge under this policy. Your profit margins would come from supplementary products, so there would be an incentive to cross-sell. If you judge by popular discourse, annoying sales and marketing seems to be the biggest issue with Swiss health insurance. [7]

This is an unintended consequence because normalising incentives for basic health insurance would expect insurers to compete in terms of service quality. Since hospital infrastructure in Switzerland is governed by the cantons, the insurers don't have any grounds to compete here.

You can propose to mitigate this by outright enforcement against unwanted sales calls. This is treating the symptom. In my opinion, the idea of selling supplementary products is a net benefit for society. The law will dictate what constitutes basic health services, but the list would and should evolve with time. That requires both technological advances and a rise in healthcare standards - what is considered essential to life. And it is helpful to have a financial incentive to develop new healthcare techniques. An attempt to curb cross-selling practices by insurers is going to stifle this innovation.

References

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- [6] Versicherungsaufsichtsgesetz (VAG), § 146; Versicherungsvertragsgesetz (VVG), § 204. See also: Finanztip, "Alterungsrückstellung in der privaten Krankenversicherung." Available at: <https://www.finanztip.de/pkv/pkv-alterungsrueckstellung/>
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